



Armada Kalamunda Group

Management of Decreased Fetal Movement Guideline

1. Purpose

This guideline aims to provide an evidence-based consensus approach to the management of woman's perception of decreased fetal movement (DFM) and consistency in the management of all women attending Armada Health Service (AHS) with reported DFM.

2. Background

Research has identified that a reduction in stillbirth rates may be achieved by increasing maternal and clinical awareness about the importance of DFM.

Stillbirths are often preceded by maternal perception of decreased fetal movement (DFM). DFM is also strongly linked to adverse perinatal outcomes such as neurodevelopmental disability, infection, foeto-maternal haemorrhage, emergency delivery, umbilical cord complications, low birth weight and fetal growth restriction (FGR). DFM for some women may be associated with placental dysfunction or insufficiency, which could lead to fetal growth restriction and/or stillbirth.

3. Definitions

There is no universally agreed definition of DFM. Maternal concern and perceived reduction in fetal movements is more important than any definition of DFM based on movement counting. Education around awareness of fetal movements recommends discussion with women to include their becoming familiar with their individual baby's movements and promptly reporting any changes.

4. Management

It has been suggested that increased maternal awareness of fetal movements (FM) is associated with a reduction of perinatal mortality and the mother's perception is considered a valuable screening tool.

- Medical and Midwifery practitioners working at AHS are recommended to complete the [Safer Baby Bundle \(NHMRC and Stillbirth CRE\) e-Learning](#) package every 2 years, to ensure they are providing women with education that is evidenced-based and best practice.
- All pregnant women booking into AHS will be provided with verbal and written information about fetal movements prior to 28 weeks gestation. Women will be advised that it is normal to perceive increasingly strong movements, episodes of movements that are more vigorous than usual, occasional fetal hiccups, and a diurnal pattern involving strong fetal movement in the evening.
- All pregnant women after 28 weeks gestation at scheduled and unscheduled maternity appointments will be provided information on the importance of maternal awareness of

fetal movements, and to report concerns of decrease in strength and/or frequency or non-diurnal pattern of movements.

- Consideration must be given to women from Culturally and Linguistically Diverse backgrounds to ensure education is given via a method appropriate to their understanding and learning needs (use of Interpreter, Brochure on DFM in appropriate written language).
- All women should be advised to contact the Antenatal Assessment Unit (AAU) or Midwifery Group Practice (MGP) midwife if they have any concern about decreased or absent fetal movements and be advised not to wait until the next day to report DFM.
- All women ≥ 24 weeks gestation who contact the AAU or advise of DFM in the antenatal clinic appointments must be advised to attend AAU for a midwifery review and completion of a DFM midwifery assessment. Presentation must not be delayed through efforts to stimulate the baby with food or drink or requesting the woman to concentrate on fetal movement patterns for a period.

Maternal concern of DFM overrides any definition of DFM based on numbers of fetal movements and women with a concern about DFM should be encouraged to contact either AAU or MGP midwife

4.1 Assessment

When a woman presents with DFM, assessment of the woman and her fetus should be undertaken as soon as possible.

Assessment Required: (To be reported on AKMR 8 Maternal Assessment Form)

- Document Maternal Obstetric History: Duration/pattern of decreased fetal movements (DFM), maternal lifestyle issues(smoking), any medication, alcohol or substance use, advanced maternal age, any abnormal abdominal pain, risk factors for stillbirth e.g. diabetes, smoking, obesity, hypertension, intra-uterine growth restriction (IUGR), congenital malformation, low PAPP-A, poor obstetric history, vaginal loss or bleeding.
- Perform baseline maternal observations (Temperature, Maternal Heart Rate, Respiration Rate, Blood Pressure, SpO₂,) including urinalysis.
- Perform an abdominal palpation, including performing a symphysis fundal height measurement.
- Assess maternal perception of fetal movement pattern with woman including history of fetal movements, frequency, strength and any changes in the pattern of movement.
- Assess for vaginal loss, uterine tightening or contractions, maternal wellbeing.
- Assess the fetal heart rate.
 - Between 20 - 23+6 weeks gestation with fetal handheld doptone.
 - Between 24 - 27+6 weeks gestation discuss with Gp Obstetrician on requirement to undertake CTG.
 - From 28 weeks undertake CTG. Ensure CTG review sticker is completed on ISP and on the AKMR 8 Maternal Assessment form on completion of CTG.

- Women presenting with DFM presentations >24wks will be reviewed by the GP Obstetrician and Specialist Obstetrician, unless it is a first presentation and there is an absence of risk factors, CTG/Fetal Heart Rate is normal, and the woman is reassured.
- The Midwife and GP Obstetrician and Specialist Obstetrician will document their assessment and plan on the AKMR 8 Maternal Assessment form and DMR. The midwife will place the AHS DFM sticker on the AKMR 8 to highlight the presentation and in the women's pregnancy handheld record.

4.2 Perform a CTG if gestation $\geq$ 28/40.

All CTG's conducted on women who are <34 weeks gestation must be signed by the GP Obstetrician or Specialist Obstetrician on the AKMR8 Maternal Assessment Form and Intellispace Perinatal (ISP)

If CTG Normal and First Presentation of DFM

With no risk factors for stillbirth/growth restriction and maternal perception of DFM resolved, reassure the woman; provide "movement matters leaflet", notify the AAU coordinator, GP Obstetrician, and then discharge home continuing antenatal care with the usual health care provider. Ensure women informed to call back to AAU if she has any further concerns in relation to DFM or any other obstetric concerns.

If CTG Normal and Repeated Presentation of DFM

With no risk factors for stillbirth/growth restriction and maternal perception of DFM resolved, reassure the woman; discuss presentation with the AAU Coordinator and notify the GP Obstetrician or Specialist Obstetrician of the presentation and requirement of medical assessment. If decision for discharge is made by the Specialist Obstetrician provide the woman with "movement matters leaflet", ensure the woman is aware of any plans made, or aware of her next appointment, discharge home with advice to call AAU if she has any further concerns in relation to DFM or any other obstetric concerns.

- GP Obstetrician or Specialist Obstetrician must document their assessment and plan on the AKMR 8 Maternal Assessment and make entry in woman's DMR.

With risk factors and/ or DFM remaining present: Arrange an ultrasound (USS) for AFI/ MVP/ UAPI/ MCA/ AC / EFW. Within 12 - 24hrs. Discuss woman with the GP Obstetrician /and/or Specialist Obstetrician and document this discussion on the AKMR 8.

If Abnormal:

Arrange urgent GP Obstetrician and/or Specialist Obstetrician review. Specialist Obstetrician plan to be documented in the AKMR 8 Maternal Assessment form and DMR. Depending on abnormality of CTG, if urgent ultrasound is requested assessment must include:

- Fetal Biometry
- Maximum Vertical Pocket (MVP) and Umbilical Artery (UA) Doppler/ MCA.
- Fetal morphology (if not previously performed)

When escalating care, e.g., reviewing bloods, USS, or CTG concerns. The reporting midwife **MUST** co-sign with the medical officer the acknowledgement of results reviewed and/or any follow up care planned.

4.3 Further management

- Attach the AHS DFM sticker in woman's handheld pregnancy record and AKMR8 Maternal Assessment Form to highlight the presentation was DFM. Record the date and gestation of the presentation for DFM and what investigations were undertaken.
- Ultrasound scan assessment for fetal biometry AFI/ MVP/ UAPI/ MCA/ AC / EFW should be considered for women presenting with no risk factors but recurrent DFM presentations within 12 – 24 hours.
- Testing for feto-maternal haemorrhage should be considered in the preliminary investigation of women with DFM, particularly if there is a history of sustained or recurrent DFM. If feto-maternal haemorrhage is suspected and immediate birth is not indicated by CTG findings, escalation must occur to the Specialist Obstetrician.
- Each DFM presentation must have a complete midwifery assessment undertaken, additional investigations dependent on the DFM presentation must be undertaken as per Appendix 1.
- Wherein the presence of a normal clinical assessment (including a CTG and ultrasound), maternal concern remains about DFM, further management should be individualised, and GP Obstetrician and/or Specialist Obstetrician review must be arranged.
- Shared decision-making on the benefits and risks of planned birth should be facilitated through the provision of verbal and written information, individualised to each woman based on her clinical situation including gestational age, woman's preferences, findings of clinical investigations and the presence or absence of stillbirth risk factors. Where possible, birth should not be planned prior to 39 weeks gestation unless clinically indicated.

All pregnant women who have an IOL for DFM MUST have cord blood gases taken at birth.

5. Legislative context

The following documents are required to give effect to this policy [i.e. the documents included are mandatory]:

- Health Department of Western Australia (HDWA) [Clinical Handover MP 0095](#)
- HDWA [Cardiotocography Monitoring Policy MP 0076/18](#)
- HDWA [Clinical Incident Management Policy MP 0122/19](#)
- East Metropolitan Health Service (EMHS) [Fetal Surveillance and Cardiotocography \(CTG\) Monitoring Policy EMHS: 195](#)

6. Supporting information

The following documents support the implementation of this policy:

- [Induction of Labour Guideline OBS-GUI-0204-15](#)
- [Recognition and Response to Acute Deterioration AKG-POL-0388-14](#)

7. Compliance, monitoring and evaluation

Compliance against this guideline will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS

Specific findings are to be reported at the Obstetrics Safety and Quality Departmental meeting for review and identification of any opportunities for improvement.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

Standard 6 – Action 6.3 Partnering with consumers.

- Standard 6: Action 6.9 – Communicating critical information.
- Clinicians and multidisciplinary teams use clinical communication processes to effectively.
- Standard 8: Action 8.6 - Escalating care

References

1. Billie F Bradford, D. J. (2023). Decreased fetal movements: Report from the International Stillbirth Alliance conference workshop. *International Journal of Gynecology & Obstetrics* , e1-e4.
2. Centre of Research Excellence in Stillbirth, M. R. (2019, September). *Clinical practice guideline for the care of women with decreased fetal movements for women with a singleton pregnancy from 28 weeks gestation*. Retrieved from Stillbirth Centre of Research Excellence: https://stillbirthcre.org.au/wp-content/uploads/2021/03/Element-3_DFM-Clinical-Practice-Guideline-1.pdf
3. Service, G. o. (2025, 06 27). *Decreased Fetal Movements*. Retrieved from Obstetrics and Gynaecology Clinical Practice Guideline: <https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/WNHS/WNHS.OG.DecreasedFetalMovements.pdf>

9. Document contact

Enquiries relating to this guideline may be directed to:

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10. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4.0	22/08/2025	22/08/2025	Review and updated entire document to latest evidence based practice.

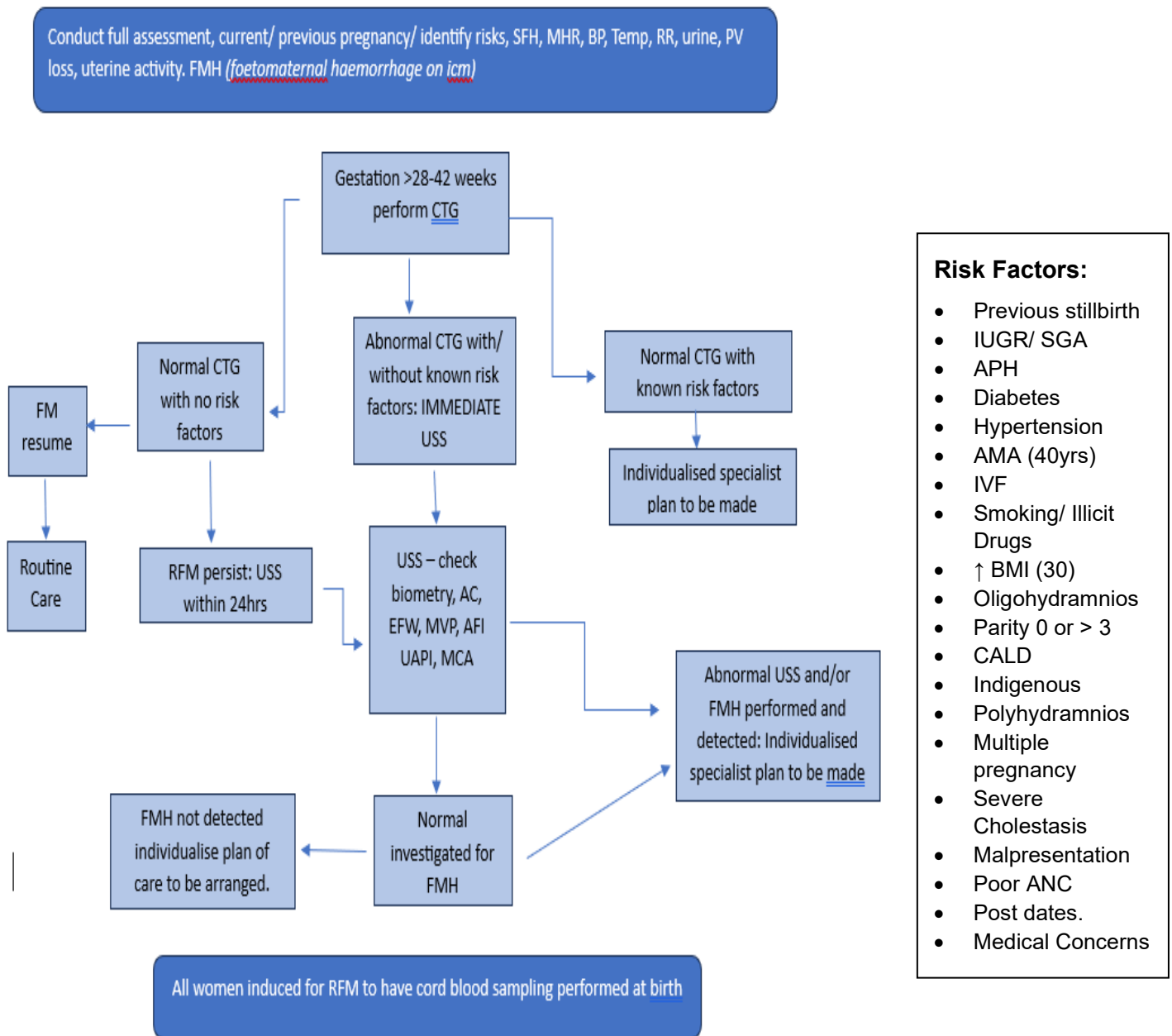
			Updated flowchart included.
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11. Authorisation

This guideline has been authorised and issued by the Director Clinical Services.

Authorised by	Dr Tania Strickland – Director Clinical Services
Authorisation date	22/08/2025
Published date	22/08/2028

Appendix 1: Flow chart for the management of decreased fetal movement > 28/40 weeks gestation



Please note: Caregivers should be aware of the increased risk of poor perinatal outcome in women presenting with recurrent RFM. There are no studies to determine whether intervention (e.g. delivery or further investigation) alters perinatal morbidity or mortality in women presenting with recurrent RFM. Therefore, the decision whether to induce labour at term in a woman who presents recurrently with RFM when the growth, liquor volume, dopplers and CTG appear normal must be made after careful consultant counselling of the pros and cons of induction on an individualised basis. (RCOG, 2017).